

Male genital edema in Crohn's disease

To the Editor: An otherwise superb account of the cutaneous manifestations of Crohn's disease is unfortunately incomplete because Thrash et al¹ have omitted genital presentations, particularly penile edema/lymphedema/granulomatous lymphangitis. Genital involvement is rare in Crohn's disease but can be encountered as genital ("knife-cut") ulcers, fistulae, fissures, abscesses, and genital edema.² In 20 years of specialized male genital clinical activity, I (C.B.B.) have diagnosed and managed the last situation about a dozen times. In some patients (over a third in my experience) it can be the first and only intimation of Crohn's disease, thus is an important pointer to gastrointestinal investigations and the confirmation or exclusion of Crohn's disease.³

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Reply: Genital edema in Crohn's disease

To the Editor: In his letter to the editor, Dr Christopher Bunker draws attention to an important clinical finding of genital edema in Crohn's disease, which we omitted from our previous CME article titled "Cutaneous manifestations of gastrointestinal disease." Dr Bunker reports several cases of male genital edema as a sole finding in Crohn's disease in his clinic. In a letter published in the August 2011 *Journal of the American Academy of Dermatology*, Reinders et al¹ also reported a case of granulomatous lymphangitis that presented as asymptomatic penile

and scrotal edema in a patient with no known history of Crohn's disease. Although colonoscopy showed no histologic abnormalities, video capsule endoscopy demonstrated ulcerations in the ileum with subsequent development of diarrhea and bloody stools and diagnosis of Crohn's disease. Thus, genital edema preceded the development of Crohn's in this patient and was an important diagnostic clue as Dr Bunker correctly points out. Several other cases have also been reported in the literature of genital edema in Crohn's disease. It is important to note genital findings are not limited to male patients, but can also present as vulval edema in female patients. We appreciate Dr Bunker bringing this to our attention.

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Tumor recurrence after Mohs micrographic surgery

To the Editor: We read with interest the article by Campbell et al¹ examining factors contributing to tumor recurrence after Mohs micrographic surgery (MMS). Although few studies have examined the role of surgeon error with tumor recurrence,² it is notable that in their study 26% (5/14) of recurrent lesions were found to have tumor at the margins of the MMS slides. Presumably, this resulted from the presence of dense inflammation on the slides, progression of actinic keratosis to squamous cell carcinoma in situ during the interval between initial and follow-up surgeries, poor slide quality, or other factors.³ Although this percentage of residual tumor at the margins may seem surprisingly high to some readers, it is similar to that of our academic institution. This letter aims to draw further awareness to these potential sources contributing to MMS recurrences. In addition, we seek to highlight that the findings of